COASTLINE FOOT & ANKLE CENTER

☆ FAXED ☆
APR 24 2026

DR. R TYSON SCOTT DPM LLC
DR. SCOTT BERG DPM

DATE: 4 . 24 . 2026

Send to: NWRA Dallas	**From: Taylor R., Medical Scribe**
Attention:	**Phone Number: 503- 370-8784**
Phone Number:	**Fax Number: 503-362-4017**
Fax Number: 503. 623. 2196	**Number of Pages (Including Cover)** 7

☐ **URGENT** ☒ **FOR REVIEW** ☐ **PLEASE REPLY**

REGARDING: L. Hazelton

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Dallas Location!
503-623-2196

Northwest Rehabilitation Associates, Inc.

PHONE (503) 371-0779 FAX: (503) 371-0886

Rehabilitation Services Referral

Patient Name: Lonny Hazelton
Phone: (503) 899-8342
DOB: 10/15/73 SSN - -
Insurance: See demos ID#: Group#:
Diagnosis: Peroneal tendinitis, sprain talofibular lig, mild extensor digitorum tendinopathy
ICD 10: M76.72, S96.112A Onset date: / /
MD UPIN:

Frequency: 1-2 times per week for 4-6 weeks

Patient Follow-up with MD on: / / ; or in _____ weeks

Physical Therapy	Occupational Therapy	Speech Therapy
☒ **Evaluate and Treat**	☐ **Evaluate and Treat**	☐ **Evaluate and Treat**
☐ Evaluate only	☐ Evaluate only	☐ Evaluate only
☒ Gait and Transfer Training	☐ Self care/ADLs	☐ Speech
☐ Functional Activities	☐ Thermal Modalities	☐ Language
☐ Therapeutic Exercise	☐ DME Eval	☐ Swallowing
☐ Wheelchair or DME Eval	☐ Cognitive Skill Training	☐ Cognitive Skill Training
☐ Thermal Modalities		
☐ Electrical Stimulation	☐ Hand splint	
☐ Therapeutic Pool (underwater treadmill)		☐ **DRIVEABLE (driver's safety training)**
☐ Women's Health	☐ **ASAP: Hand or wrist pain**	
☐ **ASAP** ☐ **Pain** ☐ **Vertigo**	☒ ASTYM Modality + Gait analysis	
☐ **WEST SALEM ONLY** (Capital Manor site)		

Physician Name: Scott Berg, DPM Date: 4/22/26
Physician Signature

Thank you for this referral! Mike Studer, PT, MHS, NCS, CEEAA, CWT, CSST

3270 Liberty Rd S. Salem, OR 97302 – Healthy Aging and Neurology
3220 Liberty Rd S. 97302 – Sports and Orthopedics
1955 Dallas Hwy NW, 97304 – Capital Manor

HAZELTON, Lonny D DOB: 10/15/1973 (52 yo M) Acc No. 122444 DOS: 04/22/2026

Coastline Foot and Ankle Center

MRI Results

Hazelton, Lonny D (DOB:10/15/1973)

Provider:Scott L Berg, DPM

52 Y Sex: Male
Phone: 503-899-8342
Address: 4022 SE EMILY WAY, ADAIR VILLAGE, OR-97330-9178

Date: 04/22/2026

Subjective:

Chief complaints:

MRI Results

History of Present Illness:

Consult:

Patient presents today for MRI Results. No further concerns were presented today.

Current Medications:

Taking Tirzepatide
Taking Sildenafil Citrate
Taking Pantoprazole Sodium
Taking Ondansetron
Taking Methylphenidate ER
Taking metFORMIN HCl
Taking Lisinopril-hydroCHLOROthiazide
Taking EPINEPHrine
Taking Colchicine
Taking buPROPion HCl
Taking Allopurinol
Medication List reviewed and reconciled with the patient

Past Medical History:

No Medical History Documented.
Medical History Verified.

Allergies:

N.K.D.A.
Allergies Verified.

Surgical History:

surgery 2/26
Surgical History Verified.

Hospitalization / Major Diagnostic Procedures:

No Hospitalization Documented.
Hospitalization Verified.

Family History:

No Family History documented.
Family History Verified.

Social History:

Tobacco Use:

Tobacco use: no. Tobacco Control (Standard): Tobacco use: Nonsmoker, Additional Findings: Tobacco non-user Current nonsmoker.

Drug/Alcohol:

AUDIT-C (Standard): Did you have a drink containing alcohol in the past

year? No, Points 0, Interpretation Negative.
Social History Verified.

Review of system:

ROS:

ROS Negative for fevers, chills, nausea, vomiting, diarrhea, constipation, headache, visual disturbance, neck pain, chest pain, shortness of breath. All other systems are negative, except as noted in the HPI

Objective:

Vitals:

Ht: 69 in, Ht-cm: 175.26 cm.

Examination:

FOOT and ANKLE:

General: Patient well appearing, in NAD. In good spirits., Alert & oriented x3.

Vascular: Capillary refill time is brisk, Dorsalis pedis, posterior tibialis pulses, normal 2+/4 bilateral.

Neurologic: Grossly intact without focal deficit.

Dermatologic: Skin turgor texture and temperature normal. No lesions or rashes noted. Web spaces clean and dry. Previous scar noted from ankle injury/ surgery..

Musculoskeletal: 5/5 Muscle strength in all 4 quadrants. ROM forefoot, midfoot, and rearfoot WNL, No gross deformities noted, Examination of the RIGHT lower limb:Moderate bunion No pain with eversion and inversion of the foot. No pain with plantar flexion or dorsiflexion of the foot. No pain with ROM of the 1st MTP joint, loaded or unloaded. No pain anterior medial ankle joint, crepitus noted, Reducible hammertoe 2nd, previous scar noted from ankle injury

Examination of the LEFT lower limb: Left most symptomatic; No pain with eversion and inversion of the foot. No pain with plantar flexion or dorsiflexion of the foot. Pain with ROM of the 1st MTP joint, loaded. Moderate bunion,; non-painful. Pain anterior medial ankle joint, pain along posterior tibial tendon, Pain along talofibular ligament. Pain anterior medial and lateral ankle joint. Pain with varus and valgus rotation.

Radiographic Exam 4/6/2026

Left AP: Moderate Bunion, Hammertoe 2-5, Otherwise, no other fractures or dislocations.

Left Lateral: Mild osteoarthritis phalangeal joint, Otherwise, no other fractures or dislocations.

Right AP: Moderate Bunion, Hammertoe 2-5, Otherwise, no other fractures or dislocations.

Right Lateral:Hardware noted in ankle joint from previous ankle fracture, Otherwise, no other fractures or dislocations.

Done at Salem Clinic on 3/17/2026

Impression: Left ankle

1. No evidence of acute osseous abnormality.

Approved by Dr. Jose Novoa.

MRI: Done at Salem Clinic on 4/17/2026

Impression: Left ankle

1. Sprain of the anterior talofibular ligament.
2. Mild extensor digitorum longus tendinopathy.
3.Peroneus longus tendinopathy.
4.Moderate ankle joint effusion.
5. Edema of the anterior process of the calcaneus. There is no evident fracture line. This could represent degenerative edema or bone contusion. There is mild edema of the adjacent proximal dorsal cuboid. There is a second region of edema in the calcaneus at the plantar aspect of the calcaneocuboid joint.
5.Changes suggesting midtarsal joint sprain (chopart sprain), inclusing possible sprain of the short plantar calcaneocuboid ligament, sprain of the ligament between the cuboid and lateral navicular, and sprain of the dorsal talonavicular ligament.

Approved by: Dr. Travis E Fromwiller.

HAZELTON, Lonny D DOB: 10/15/1973 (52 yo M) Acc No. 122444 DOS: 04/22/2026

Assessment:

Assessment:

Idiopathic gout, left ankle and foot - M10.072 (Primary)
Other instability, left ankle - M25.372
Pain in left ankle and joints of left foot - M25.572
Bunion of right foot - M21.611
Osteoarthritis Left ankle and foot - M19.072
Peroneal tendinitis of left lower extremity - M76.72
Strain of muscle and tendon of long extensor muscle of toe at ankle and foot level, left foot, initial encounter - S96.112A
Bunion of left foot - M21.612

Plan:

Treatment:

Peroneal tendinitis of left lower extremity

Start Meloxicam Tablet, 7.5 MG, 1 tablet daily for 2 weeks then as needed., Orally, Once a day, 30, Refills 0

Notes: Discussed with Lonny the nature of their current symptoms. Discussed the patient's MRI results in great detail. I related the MRI findings to their physical symptoms. Patient expressed understanding and all questions were answered to satisfaction. At this time I expressed his symptoms associated with peroneus longus tendinopathy, sprain of the anterior talofibular ligament, and edema of the anterior process of the calcaneus is where he is most symptomatic. Discussed conservative treatment options such as custom orthotics, cortisone injections, Physical therapy, and NSAID's. Patient expresses he would need to consult his PCP to take meloxicam due to stomach issue history. I explained that the orthotics can help place the feet in proper position. Orthotics can also evenly disperse pressures in the feet, alleviating some pressure in the area of concern. Patient expressed understanding and elected to try custom orthotics and may return to have them scanned. Patient expressed understanding and all questions were answered to satisfaction. He elected to go to PT, will call insurance about orthotics, cortisone injection, and wants a pulse dose of Meloxicam but will contact his PCP before taking it. At today's visit, a 3 cc mixture of 2% **lidocaine and Kenalog-10 was administered to the anterior medial ankle joint of the left foot.** A referral will be sent to Dallas NWRA. Pulse dose will be sent to Fred Meyer in Corvallis.

Bunion of left foot

Notes: Radiographic images were reviewed and findings are consistent with a bunion deformity on his left with pain. Physical exam correlates symptoms with radiographic findings. Reviewed the nature of bunion deformities and associated symptoms. Reviewed conservative treatment options including wider shoe gear as well as custom orthotics to provide proper mechanical support and alignment to the lower limbs. Patient understands that this will not fix the deformity but rather slow its progression. Explained that the deformity may be fixed with surgical correction. We discussed the nature, risks, benefits, and recovery process of the procedure in detail. Also reviewed the permanent limitations of certain motions and activities that will follow after surgery. Modifications to compensate for this lack of motion were reviewed with the patient. The patient expressed understanding and all questions were answered to his satisfaction. Patient elected to do conservative methods that also align with conservative treatment for his tendinopathy.

Portions of this chart were written by Taylor Russell, Medical Scribe, with oversight by Scott Berg, DPM. I have reviewed the information recorded by the medical scribe for accuracy and agree with its contents.

Scott Berg, DPM

Follow Up:

2 Weeks (Reason: f/u from PT and Cortisone inj)

HAZELTON, Lonny D **DOB:** 10/15/1973 (52 yo M) **Acc No.** 122444 **DOS:** 04/22/2026

Billing Information:
Procedure Codes:

J3301 INJ TRIAMCINOLONE ACETONIDE 10 MG. Modifiers: LT
20605 DRAIN/INJ, JOINT/BURSA intermed w/o us.

This encounter has been **Marked as Done**

Electronically signed by Provider: Scott Berg on 04/23/2026 at 10:26 AM PDT
Sign off status: Completed

Provider: Scott L Berg, DPM

Enc date: 04/22/2026
Generated for Printing/Faxing/eTransmitting on: 04/24/2026 08:01 AM PDT

Patient Demographics

Patient Name: Lonny Hazelton D

Street Address: OR

Mailing Address: 4022 SE EMILY WAY ADAIR VILLAGE OR 97330-9178

Home Phone: 503-899-8342

Work Phone:

Date of Birth: 10/15/1973

Social Security Number:

Email Address: lonny.hazelton@outlook.com

Insurance Information

Primary Insurance: Providence Collective Health Phone Number: 855-383-3585
PO BOX 23055, TAMPA, FL, 33623-2134

Subscriber Name: Hazelton, Lonny D Date of Birth : 10/15/1973
Subscriber ID: P2040769901 Group Number: 0261896

Secondary Insurance: Tricare West Region
PO BOX 202112

Subscriber Name: Hazelton, Lonny D
Subscriber ID: 00635624400

Emergency Contact Information

Emergency Contact Name: Phone Number:
Pharmacy Name: FRED MEYER #701070 Pharmacy Number: 541-754-5583